



Why and how do workplaces invest in mental health and wellbeing? A systematic review and process tracing study

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ABSTRACT

In recent years, investment in workplace mental health and wellbeing has grown. However, there is little understanding of how resource allocation decisions are made in such settings. To ensure evaluative research can support resource allocation, a process-based understanding of decision-making would be helpful. In phase one a systematic review of the literature on the implementation of workplace mental health and wellbeing schemes was conducted to draw insights on workplace resource allocation processes. In phase two an in-depth case study of a large company was conducted with interviews with those involved in resource allocation and wellbeing. Interviews were coded and analysed using descriptive and explanatory accounts. The findings from the review and case study were combined and developed into a causal process theory. This study shows that the stages in mental health and wellbeing investment revolve around ensuring organisational buy-in, workforce investment, and continual evaluation. Further work is needed to explore the transferability of the resulting process theory across different types of workplaces. However, it is clear that the features of real world decision-making in workplaces present challenges and opportunities for the research community.

1. Introduction

Workplace mental health and wellbeing is an area of increasing concern in the UK and around the world, with public awareness of the issue growing (Deloitte, 2017). Depression remains the leading cause of disability worldwide, and other common issues, such as anxiety, contribute to presenteeism – reduced productivity while at work – and absenteeism – habitual non-attendance (Evans-Lacko and Knapp, 2016; Friedrich, 2017; Wu et al., 2021). In the UK, Deloitte (2024) estimate that sickness absence rates (absenteeism) have risen by 20% since 2020/21, costing around £7 billion annually. Meanwhile, staff turnover driven by poor mental health incurs costs of around £20 billion annually, and presenteeism presents a greater challenge, with costs at £24 billion per year. Combined, poor mental health costs employers an estimated £51 billion in 2023 (Deloitte, 2024). The Centre for Mental Health estimates that these costs to industry are significantly higher, totalling £100.6 billion, with the largest costs stemming from staff turnover (£43.1 billion) and presenteeism (£41.8 billion) (Centre for Mental Health, 2024). Presenteeism occurs due to a range of factors, such as financial stress or job dissatisfaction (Centre for Economics and Business

Research, 2023).

With individuals spending on average two-thirds of their waking hours at work, the workplace is one of the best-placed, and most convenient, environments for promoting the wellbeing of individuals (Chetty, 2017; Quirk et al., 2018). Promoting mental wellbeing through a safe environment, consisting of productive and healthy working conditions, was the subject of the National Institute for Health and Care Excellence (NICE) guidelines on mental health in workplaces in the UK (NICE, 2009). The Chartered Institute of Personnel and Development (CIPD) has shown however that, “despite an increasing number of workplace health and wellbeing services being put in place, employees have an increasing number of mental health issues” (CIPD, 2023). Evidence on the cost-effectiveness of interventions is needed if scarce resources devoted to improving workplace mental health and wellbeing (MHWB) are to be allocated efficiently and research on workplace wellbeing investment has often centred around establishing intervention (cost-) effectiveness.

To date, there has been relatively little research exploring how and why workplaces choose to invest in mental wellbeing. One of the reasons that health economists have widely adopted cost-utility analysis (rather

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than the more traditional methodology of cost-benefit analysis) is that the approach is deemed to align with decision-maker objectives in the health care system to maximise health from a fixed budget (Coast, 2009). It is important not to start by making assumptions about the objectives of employers and workplaces; doing so may result in the wrong choice of methodology or the wrong choice of how to present and disseminate evidence. Understanding how and why decision makers in workplaces allocate resources to MHWB initiatives in the real world is therefore a necessary first step, as it forms the foundation for ensuring that resources are used in a way that can effectively support employee wellbeing (informed by appropriate and accessible evidence). Before exploring intervention strategies that improve mental health outcomes, it is crucial to investigate the underlying motivations and frameworks guiding resource allocation.

1.1. The organisational model of decision-making

Health economics is the study of the allocation of scarce resources for health. The main way in which the health economics discipline seeks to support efficiency and equity is via the production of economic evaluation evidence (Drummond et al., 2015). Economic evaluations of workplace mental health interventions are starting to be conducted and provide decision-makers with potentially useful information on the relative value-for-money of different intervention strategies (Nogues and Finucan, 2018). However, as noted above, the decision-making model implicit with the production of an economic evaluation is a rational agent comparing alternative intervention strategies to maximise health benefits from a fixed budget.

Early work on the behavioural theory of firms suggests that given bounded rationality and internal conflict, employers are likely to satisfice (choose an acceptable option) rather than try to make optimal decisions (Cyert and March, 1963). This foundational work has influenced subsequent models in organisational decision-making, which now encompass cognitive strategies and governance mechanisms to overcome uncertainty and conflict (Grandori, 2001). Similarly, models of the policy process aim to simplify decision-making within complex organisational contexts. The policy stages heuristic, for example, breaks a process into stages, with evidence gathering (evaluation) following the implementation of a policy (Sabatini and Jenkins-Smith, 1993) as opposed to informing it as assumed in a rational choice model.

In generating evidence, research has largely focused on the clinical effectiveness of workplace wellbeing for employees and cost-effectiveness to employers, particularly post-COVID-19 pandemic (Bondar et al., 2022). Currently, relatively little is known about the organisational context and processes for these decisions—how decision-makers identify and analyse alternative strategies, make budgetary decisions, and align these with organisational goals.

1.2. Policy context

UK policymakers have emphasised the need for workplaces to invest in mental health programmes, but beyond the *Mental Health Toolkit for Employers* (2016), there is limited guidance for organisations on how to invest (Work Foundation, 2019). Many employers are dedicating considerable resources to employee wellbeing, but they could see a greater return on investment by investing in a more systematic way (CIPD, 2023). It has been suggested that the rationale for investing in employee wellbeing is clear, but there is ongoing debate regarding exactly how workplaces should invest (What Works Wellbeing, 2017).

The influential Stevenson and Farmer (2017) report acknowledges the inadequacies of a one-size-fits-all approach to workplace wellbeing. They emphasise how the needs and capacities of organisations can vary significantly based on their size and employee demographics. Small and medium-sized enterprises (SMEs), as well as the self-employed, are encouraged to start by utilising local organisations and networks, rather than looking to implement complex mental health strategies (Stevenson

and Farmer, 2017). Conversely, larger organisations are expected to have greater resources and structures in place, making it theoretically easier for them to adopt comprehensive mental health standards. It should be easier for large organisations to invest in bespoke wellbeing activities due to their pre-existing scale and structures, and an ability to spread costs over potentially tens of thousands of employees. However, even in large workplaces, decision-makers still face difficulties gauging the mental health and wellbeing needs of their employees because of the stigma surrounding mental health conditions (Wu et al., 2021).

Workplaces have various routes available to implement employee wellbeing programmes and must consider different factors in their decision-making process. This includes conducting comprehensive needs assessments by utilising employee surveys and health data, providing training and education to staff, and fostering a supportive environment. Organisations' successful uptake of new ideas and strategies hinges on their decision-making process; the input they receive from staff; the types of information they use; and their ability to create a culture that prioritises employee wellbeing. Hence, mental health and wellbeing represent a growing area of concern both for policymakers and employers. First, we need to understand the processes used by workplaces to make resource allocation decisions, before considering how best to implement effective interventions. Uncovering how workplaces make resource allocation decisions will lay a foundation to inform the development of more impactful mental health and wellbeing strategies in the future. The aim of this study was to investigate why and how workplaces invest in mental health and wellbeing.

2. Methods

2.1. Study design: overview

This study used a qualitative approach, combining a systematic review of the literature with in-depth case study work. Process tracing methods (Beach and Pedersen, 2019) were used to synthesise the findings across four stages: **problem identification**, to explore how MHWB issues are recognised; **policy formulation**, examining how resources are allocated to address these issues; **policy implementation**, analysing the execution of MHWB strategies; and **evaluation**, assessing the effectiveness of these interventions. The combination of systematic review and empirical case study allowed us to build upon prior research and a real-world context to explore decision-making processes and resource allocation in detail. This approach drew on the policy-making literature, and aimed to embed data triangulation and enhance the external validity of our findings.

2.2. Systematic review

The research question underpinning the systematic review was: how do workplaces allocate resources to mental health and wellbeing interventions? This paper uses the terms: 'interventions', 'programme(s)', 'initiatives' and 'strategies' interchangeably to describe types of wellbeing investments.

To structure and organise this review, the Preferred Reporting Items for Systematic Reviews, and Meta-Analyses extension for Scoping Reviews (PRISMA-SCR) were followed, in particular, the guidelines developed by Tricco et al. (2018). For inclusion studies must have discussed information on either: (1) budgets or spending; (2) the (human, financial, or physical) investment decision-making process, or (3) barriers affecting resources being mobilised, including resource availability. Empirical studies, discussion papers and previous systematic reviews written in English were eligible for selection. The final electronic search was conducted on March 18th, 2022, using four prominent databases: PubMed, Scopus, Web of Science, and PsycInfo, chosen for their relevance to the research topic. The search strategy and keywords were selected based on the inclusion criteria and the research question. Search terms were used to identify papers reflecting four categories: (i)

workplace setting, (ii) mental health and well-being focus, (iii) investment decision, and (iv) interventional focus (full search string included in [Appendix 1](#)). For inclusion, studies had to include at least one search term from each category in the title or abstract.

The records yielded from each electronic database were added to an EndNote (version 20) library and combined with the key studies, in addition to studies identified and recommended by experts. Records were then de-duplicated. One reviewer (LH) screened each record at the title sift and abstract stage, per the eligibility criteria. Two independent reviewers (RJ and HA), blind to the results of LH, reviewed a sample of 5% to ensure consistency. We had a group discussion about any discrepancies and then repeated this for the shortlist of studies screened at the full-paper stage. Data pertaining to the study characteristics and content (motivations and processes for mental health and wellbeing investment) were extracted from included articles and entered into Microsoft Excel. LH, RJ and HA were involved in screening and data extraction.

2.3. Case study

2.3.1. Case study selection

A principal objective of case study research is to investigate in depth a set of decisions, revealing the factors influencing their adoption, implementation, and subsequent outcomes ([Schramm, 1971](#)). Our study aimed to follow up on the findings from the systematic review by investigating the specific decision-making process within a ‘high investing’ workplace. To investigate the decision-making process, we used a process-tracing approach ([Beach and Pedersen, 2019](#)). We intended to use process tracing to unpack the key episodes in the process of allocating resources to initiatives, and understand the causal mechanisms that drive these decisions. Specifically, we sought to identify the factors influencing decision-makers, the sequence of events leading to resource allocation, and how these decisions align with organisational priorities and objectives. Through process tracing, we aimed to map out the interplay between individual actions, organisational structures, and external pressures, providing a detailed account of how and why certain initiatives receive investment over others.

In early theory-building process tracing, researchers construct a theory based on a ‘typical case’ or multiple cases and then test it across varied characteristics ([Punton and Welle, 2015](#)). Our case study focused on understanding factors influencing workplaces’ investment in mental health and wellbeing, facilitating initial theoretical propositions. To ensure an information-rich typical case, we sought to recruit a large workplace, which we defined as having more than 250 employees ([Stevenson and Farmer, 2017](#)). To identify candidate workplaces, we sampled organisations: (i) with workplace wellbeing awards; (ii) that were university stakeholders; and (iii) that were affiliated with the local Chamber of Commerce, using a checklist to determine whether the organisation was a high investor. Desk research was conducted using publicly available information including the number of employees, their wellbeing policies, any engagement with wellbeing toolkits, and any wellbeing awards.

The resulting case study was a large, diversified company active across multiple industries, that employs over 10,000 people in England, and over 250,000 worldwide, most of which are considered highly skilled. They also had an existing connection to the University of Birmingham and were open to exploratory research. Moreover, the heterogeneity of their principal business divisions increased the richness of the data for this theory-building stage.

2.3.2. Data collection

The main process for collecting data was through semi-structured interviews, which were to be supplemented and supported by document analysis ([Yin, 2017](#)). Interviews used a topic guide, with a set of prompts to explore various hypothesised aspects of the resource allocation process based on the theoretical literature and the systematic

review (see [Appendix 3](#)). Desk research on the organisation’s wellbeing strategy informed the development of specific questions. Interviewees were prompted to suggest documents outlining elements of the organisation’s investment process.

Interviewees were found through the key contact within the organisation (the Health and Wellbeing manager). A snowballing approach was used to find further contacts ([Kirchherr and Charles, 2018](#)). Individuals were sent the information sheet, topic guide and consent form before the interview took place. Our target was to sample until data saturation was achieved ([Saunders et al., 2018](#)). Interviews were conducted online via Zoom/Microsoft Teams where the inbuilt software was used to record the interview, and the files and transcripts were then uploaded to the secure online OneDrive where access was restricted to project members.

2.3.3. Data analysis

Data collection and analysis took place concurrently; thus, the questions in the topic guide were updated to reflect emerging findings identified through field notes. Following the conduct of interviews, open coding was applied to case study interviews in NVivo software ([Strauss and Corbin, 1990](#)). Codes were then grouped into an axial coding framework for inductive theory development ([Scott and Medaugh, 2017](#)). Following this, a descriptive account was produced classifying interview data under each category (and code within), supported by free text notes on the meaning in relation to the research question. A process-based explanatory account was then generated, covering the context, causes, and temporal processes. This account provided an understanding of the reasons behind events and the processes of decision-making. Documents (both publicly available and others shared with the research team) were analysed systematically, and a pre-defined extraction form was used to assess their content and credibility. The findings from the systematic review and case study explanatory account were then synthesised into a policy process, drawing on [Sabatier and Jenkins-Smith \(1993\)](#) ‘stages heuristic’ for conceptualising the four elements of the policy process (given the predominance of empirical themes related to these elements).

3. Results

3.1. Systematic review: included papers and characteristics

A total of 3,217 articles were identified from the electronic literature search and were added to 8 studies identified opportunistically by the study team. After de-duplication, a total of 1,760 articles were screened at the title and abstract stage and 81 articles were sought for retrieval at the full-text screening stage. Initially, five articles could not be retrieved and were all requested from the author(s) (via ResearchGate) and directly from the publishers. One additional study was recommended for inclusion following the 5% screening exercise, but this was subsequently not included following discussion between the authors. In total, 79 articles were assessed for eligibility at this stage, and 21 were included in the final review (see PRISMA flow chart in [Fig. 1](#)).

The review included eight empirical research studies, seven discussion papers, three systematic reviews, and three narrative reviews. The majority of the studies (16) were qualitative, with three adopting quantitative methods, and two using mixed-method approaches. Country settings included the UK (5), the US (3), Australia (3), Canada (1), and a mix of countries (4), with 5 studies not specifying a country. 6 studies focused on healthcare organisations, 2 on non-healthcare organisations, and 8 on multiple workplaces, with 5 studies not specifying a specific setting (shown in [Appendix 2](#)).

3.2. Systematic review findings

This section offers a summary of the study characteristics, and the key themes related to investment motivations, decision-making, and

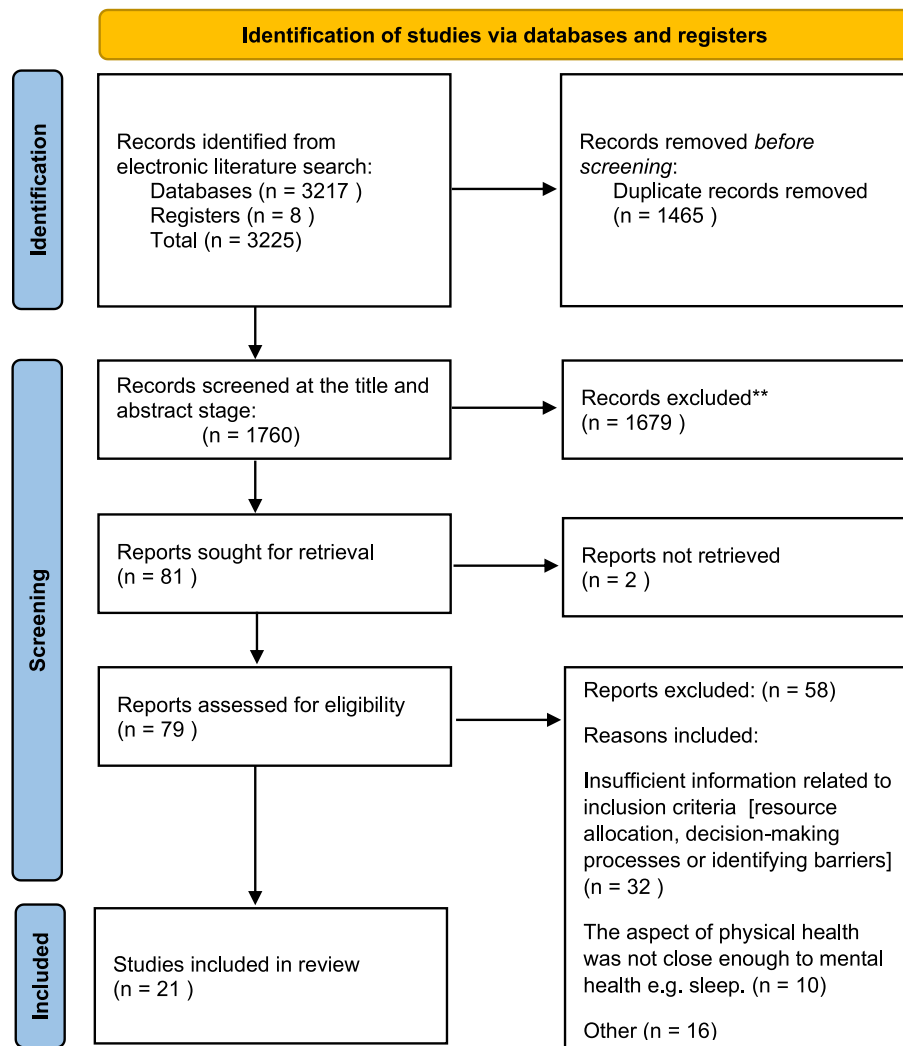


Fig. 1. PRISMA flowchart of studies selected for inclusion.

investment processes.

3.2.1. Motivations for addressing mental health in the workplace

The main motivation for addressing mental health in the studies was the intrinsic undesirability of poor mental health, both for individuals and organisations. Twelve studies identified negative outcomes resulting from poor mental health. Wieneke et al. (2019) highlighted employer concerns over healthcare costs as a significant driver for resource allocation, yet no study outlined a clear decision-making process for identifying problems. Furthermore, Wieneke et al. (2019) noted that many participants viewed mental health as a personal responsibility, making it difficult for organisations to mandate participation in well-being initiatives.

Two studies emphasised leaders' legal compliance and duty of care as key motivations, indicating that organisational actions were often driven by regulatory requirements rather than moral obligation. While Miller and Haslam (2009) noted that legal compliance drove most health activities, it often only justified minimum interventions, with economic evidence needed for more extensive programmes.

3.2.2. Staffing implications

Implementing in-house mental health and well-being schemes often required organisations to allocate staff or hire new employees, which could be costly. Three studies noted that existing staff sometimes take on additional roles, such as well-being champions, although in some cases,

these initiatives were not well understood (Wieneke et al., 2019). Fewer studies discussed staffing implications directly arising from these interventions; instead, a number of studies highlighted existing staff shortages and competing priorities as barriers to effective implementation. Overworked staff often neglect well-being responsibilities, leading to high turnover and diminished opportunities for employees to discuss mental health issues.

3.2.3. Leadership

Commitment from senior leaders was seen as crucial in organisational decision-making. Ten out of 19 studies reinforced the importance of senior leadership in resource allocation. Strong leadership was seen as essential for providing HR managers with the necessary resources and assessing intervention effectiveness (Mellor and Webster, 2013). Without visible commitment, employees would lack buy-in to well-being programmes. Leadership behaviour shaped organisational culture and inclusive decision-making could enhance intervention effectiveness (Badu et al., 2020). Conversely, criticism of mental health initiatives by leaders was seen as something that could negatively impact the work environment (Wu et al., 2021).

3.2.4. The use of evidence

Evidence-based strategies offered practical guidance for decision-making. Nogues and Finucan (2018) found that the external validity of economic evaluations was limited due to the diversity of workplace

environments. Thus, they noted organisations should consider collecting their own data for evidence. None of the studies indicated that interventions were guided by data. However, [David et al. \(2021\)](#) noted a reliance on anecdotal evidence and intuitive arguments rather than research evidence.

3.2.5. Resource allocation

[Mellor and Webster \(2013\)](#) provided detailed insights into resource allocation, noting that significant budgets were allocated to well-being initiatives, reflecting strong senior leadership commitment. However, the exact financial commitment to these programmes was unclear, particularly in healthcare settings, where budgetary control may be limited.

3.3. Case study results

14 interviews were conducted between March 2023 and January 2024. Interviews lasted between 40 min and 1hr 15 min. The organogram ([Fig. 2](#)) shows the job titles for the individuals interviewed and who they directly report to in the organisation.

138 codes were generated for the interview material through open coding. Following the descriptive account these were collapsed into seven themes for the explanatory account covering issues related to the motives and processes related to the case study's investment in mental health and wellbeing (see [Table 1](#)).

The thematic analysis from the case study and the systematic reviews were then synthesised with one another, using concepts from the policy literature to develop a process-based explanatory theory of mental health and wellbeing investment in workplaces. This is presented in [Fig. 3](#). The explanatory theory uses the policy 'stages heuristic' to structure the mechanism that leads to MHWB investment ([Garland and Beach, 2023](#)). The stages are evidenced in detail from problem identification (section 3.4) through to policy evaluation (section 3.8) drawing on a synthesis of narratives from the interviews, documentation, and the systematic review. The process theory is causal in the sense that if leaders perceive employee mental health as something that requires

Table 1

Themes from case study interviews.

Label of theme	Meaning and content
Organisational Management and Decision-Making	How decisions are made, e.g. top-down, bottom-up, and role of senior leadership.
Work Environment and Wellbeing	Workplace culture and individual wellbeing, e.g. openness and compassion.
Data Collection and Analysis	Procedures used to collect and analyse data (health data and feedback on wellbeing initiatives), e.g. surveys.
External Influences and Stakeholders	External influences on decision makers e.g. consultative forums, trade unions, customer demands.
Human Resource Management	Employee engagement and development e.g. training, learning targets, mentoring.
Business and Financial Aspects	Managing resources (financial, space, time/people) e.g. budget and value for money.
Health and Safety, and Governmental Compliance	Mandated policies the organisation has to abide by e.g. Health and Safety Executive and government regulations.

action, then this provides the impetus to formulate a policy/strategy to address employee mental health. For a strategy to be adopted it then requires 'buy-in' followed by a workforce to be implemented. The workforce will then deliver wellbeing initiatives but these initiatives will only be retained by the organisation if they are deemed successful (in this case they can demonstrate high take-up and value-for-money). The box to the right of the process theory shows the observable manifestations of the process theory in empirical accounts.

3.4. Stage 1: Problem identification

Central to any process is understanding why the problem (in this case worker mental health) came onto the agenda of workplaces. 'Problem identification', was described in both the systematic review and first case study in terms of motivations for mental health and wellbeing investment in workplaces. Synthesising across the review and empirical work, we identified three ways in which the problem enters the

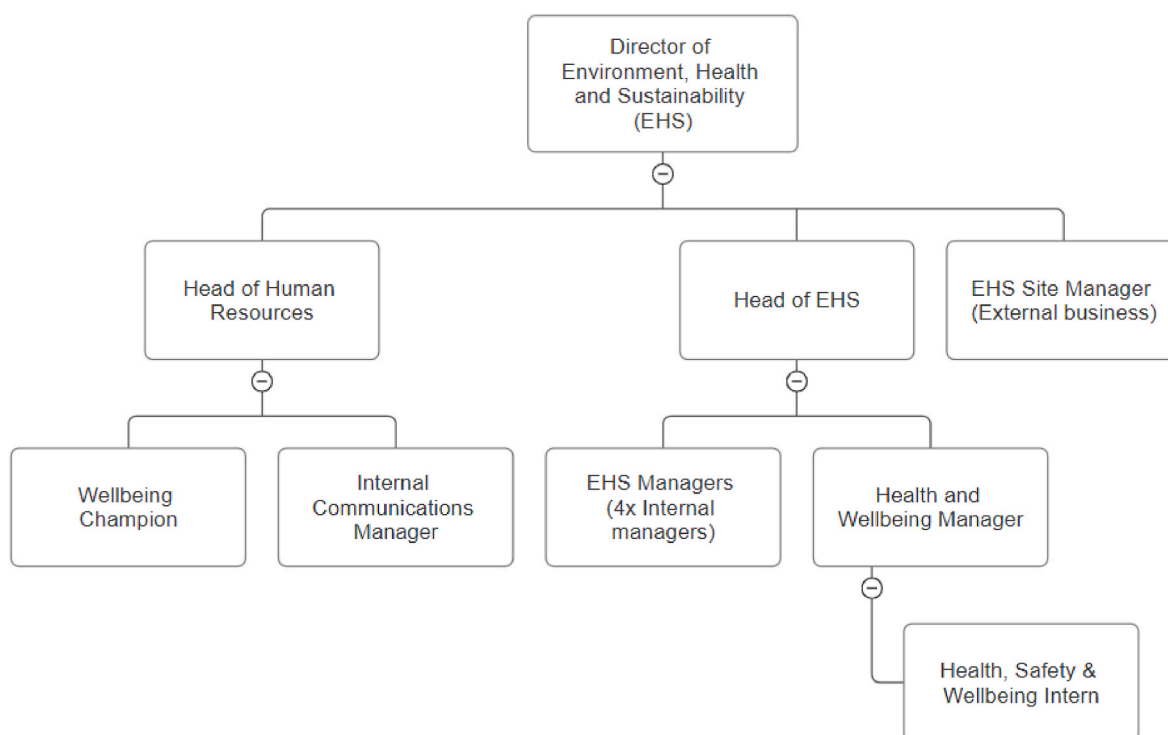


Fig. 2. Organisational organogram detailing interviewees' job roles.

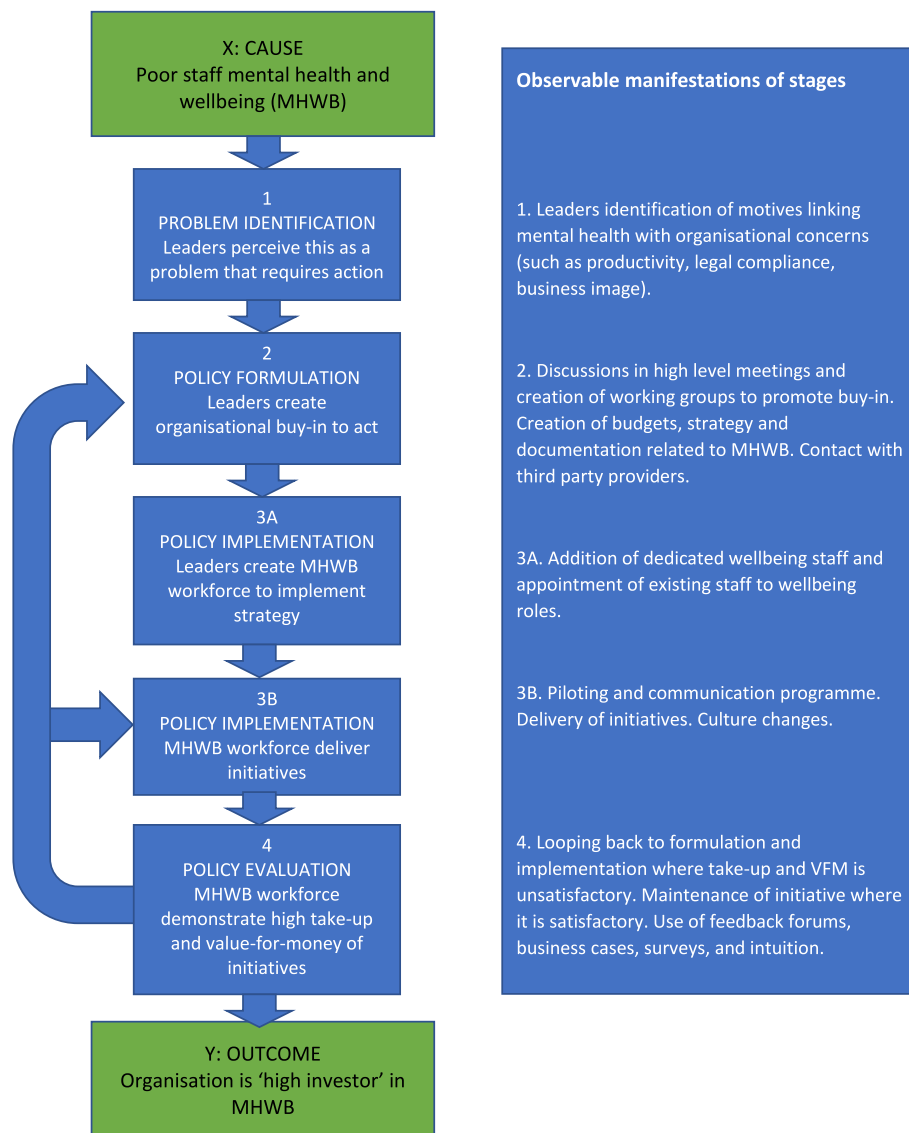


Fig. 3. Workplace MHWB investment policy stages heuristic.

employer's agenda: (i) regulation; (ii) business image; and (iii) productivity.

3.4.1. Motive 1: Regulation

In the review, legal compliance and a duty of care for employee mental health and wellbeing emanating from regulations was a key motivation. Two of the 22 studies noted that senior leaders reflected on legal compliance and legal duty of care as a critical part of their role at the organisation (Jimmieson et al., 2021; Miller and Haslam, 2009). In these cases, the organisations' primary motivation for dealing with the mental health and wellbeing of its employees is because it has been necessitated by the government or regulators, rather than due to a sense of moral obligation. Miller and Haslam found legal compliance represented the driver for most employee health activities, but only justified the minimum that was expected in terms of interventions offered. For health and wellbeing programmes to be extended beyond the bare minimum, economic evidence was necessary; "higher-level activities and resources do require a business case" (Miller and Haslam, 2009).

In the case study, workplace mental health and wellbeing did not command as much time or resource as physical health and safety components of wellbeing. Gradually, mental health and wellbeing found a place within regulations, particularly with the inclusion of wellbeing in

the Health and Safety Executive. Stress was acknowledged as a risk factor, necessitating organisations to evidence the ways they manage it:

"If you look at traditional health and safety, it is covered by COSHH (Control of Substances Hazardous to Health) Regulations, that was the stronger side of it. Now there is recognition that the health part of it encompasses mental health, financial health etc." [Head of EHS]

Psychosocial risk is probably the big one the business is dealing with right now, you know [...] but because UK is quite advanced in that, because of the health and safety executive, and the 5 steps to stress management, we knew it was coming for quite a while." [EHS Director]

For EHS Manager 2, the primary motivator for investment was the legal duty of care, emphasising the importance of avoiding legal repercussions first and foremost and then dealing with on-top interventions afterwards:

"Generally legal, like legal duty of care, is typically like number one, because you don't want to end up in hot water." [EHS Manager 2]

While being mindful of regulators, the organisation actively collaborated with various government agencies and played a role in shaping policies.

"Clearly, we have the Regulators in the UK, we're always mindful of them. Obviously, we do quite a lot of work with different government agencies and the Government itself. So again, some of that is quite key on sort of shaping, and us wanting to shape indeed, policy in the UK." [EHS Director]

3.4.2. Motive 2: Business image

In addition to adhering to existing regulations, decision-makers within the case study recognised the importance of anticipating industry trends to avoid playing catch-up and to reinforce its reputation as a forward-thinking industry leader.:

"The organisation does prioritise fulfilling legal obligations, but we also strive to be ahead of the curve and identify trends and potential issues before they become legal obligations." [EHS Manager 3]

Beyond legal considerations, decision-makers understand the significance of maintaining the organisation's image as a market leader. EHS Manager 1 highlights the value placed on being ahead of the curve both internally and externally:

"The easiest argument, and certainly what the senior leadership team like, is if you can demonstrate that we'll be ahead of the curve, not just in the UK but compared to external counterparts. Internally, it's always wanting to be ahead." [EHS Manager 1]

This commitment to staying ahead reflected a proactive approach to compliance and industry trends. To appeal to customers and enhance their chances in tendering processes, the organisation ensured it held the necessary mental health accreditations:

"When you're tendering for work, organisations often require certain accreditations [...] Mental health and wellbeing initiatives are gradually working their way into various accreditations." [EHS Manager 2]

The Head of EHS, suggested that motive 2 should be extended to include aspects of business image linked to staffing and retention.

"If you want to attract and retain people you have to have things in place to look after people's mental health. Especially staff retention. Where you have motive 2 [Business image and proactive compliance] for business image and customer influence, I would extend that to staff retention you know how we market ourselves in general, you know your glass doors." [Head of EHS]

3.4.3. Motive 3: Engaged and productive workers

The connection between employee mental health and job performance provided a third motive for investing in employee wellbeing. While recognising the challenge of precisely evidencing this link in the data, the senior leadership team held a belief in the positive impact of a happy and engaged workforce on productivity:

"We understand quite clearly the links between engaged, healthy, and productive employees. You know it's very clear. We know that healthy and safe employees who are well engaged are more productive. So, there is a clear understanding, an acceptance that if you get things right for employees, they're going to work better for you [...] there's a long-held understanding with the leadership team that this does matter." [EHS Director]

"If someone is in a poor mental health state, they are present even if they are not producing. You can't have difficulties always getting the data to show that they will be much more proactive, or they would be much more productive. It's really tough to get that data, and say, if I gave this intervention to someone then they would 20% more productive." [Head of EHS]

Wellbeing staff conveyed how they relayed health data back to wellbeing champions to help them understand the detrimental effect of sickness absence on staff and the organisation. Data from an internal

presentation to staff, shared with the research team, evidenced how while it is difficult to establish links between interventions and productivity, it is possible to convey how poor mental health leads to time off work:

Workforce lost time due to sickness per year is approx. 9024 days, which equates to 34.7 years lost time and Mental ill health accounts for 22.5% of sickness absence. [Staff presentation]

This was echoed in the findings of the review. Twelve of the studies recognised how poor mental health affects individual and organisational effectiveness. A primary motivation for organisations addressing mental health in the workplace was that employees should not be exposed to putting their mental health at risk due to involvement in the workplace. Organisations are being motivated to safeguard employees from the risk of poor mental health because it leads to poor outcomes (both directly for staff and the productive capacity of the organisation), and because they feel they must protect their staff (whether it is a legal responsibility or not).

Documents shared with the research team by the case study organisation also show that ill mental health issues represent a considerable financial impact on the organisation through absenteeism, evidenced by sick leave data. Managers reflected on how this made a case for investing more resources to employee wellbeing, to overcome sickness absences.

The EHS manager was clear that although motives 1 and 2 had long been the primary motives for investment, the organisation now wanted to enrich the lives of its employees and to do this it needed to go beyond zero harm, (where the regulation and legal compliance side usually stops). Hence, motive 3 was described as the key motivation for their investment in staff wellbeing:

"We want work to be an enricher on people's lives. So yes, we have health and safety aspects to consider, that's quite tight and comes into a legislative framework anyway, but otherwise, on the UK side we want to operate where people are coming to work and it's a positive influence on their lives. So, we want to go beyond zero harm, [...and] be making mental improvements." [Head of EHS]

3.5. Stage 2: Policy formulation

This stage focuses on how policies are arrived at, agreed upon, and communicated (Buse et al., 2012). In the context of this study, we particularly focus on the role of leadership and management in driving the agenda following the problem identification. In the case study, high-level brainstorming would occur within the senior leadership team, where a small number of board members would discuss ideas. Then, larger working groups that include wellbeing champions and mental health first aiders would be involved. Importantly, the formulation stage was an iterative process and dialogue between high-level decision-makers and those interested in the area.

"Our senior leadership team is really, really kind of passionate; they want to hear difficult questions, and they don't want us to hide difficult questions from them all." [Internal Communication Manager]

This enabled employees to become more engaged with wellbeing programmes as they see it as a priority for management. Without a clear drive from the top, it was felt that take-up was unlikely to be maximised. EHS Manager 4 was clear that understanding this drive and the role of the management team was crucial to understanding how decision-making worked in the organisation:

"[...]it comes from the drive from the top ... that's where cultural change is a big, big factor for the business in the want to do it." [EHS Manager 4]

However, management also promoted changes by incorporating new policies to hold line managers to account. Having a culture that promoted speaking up, openness and engaging with wellbeing programmes was fundamental. Additionally incorporating wellbeing into the policies

that underpinned the work of line managers, forced them to ensure they were safeguarding employee wellbeing:

"It's part of your planning that you check you are doing what you planned on, so that you've got reviews for mental health and that's when you go back to your key performance indicators and say we're doing this, is it doing what we wanted it to [...] and then that is where it becomes an embedded topic" [EHS Manager 3]

Engaging the right, experienced people during the policy formulation stage can save both time and money. Wellbeing managers, for instance, have a keen sense of what will and won't resonate with their teams. When there is a disconnect between the senior leadership team and employees, ideas that are unlikely to be well received are more likely to be suggested. However, these ideas are unlikely to progress to the policy implementation stage:

"There are lots of forums where you can bring ideas to the table. Some of them will be, "We've tried something like that before, that's not going to go well." Or, "Oh, I've not heard about that, sounds interesting." [EHS Manager 2]

Hence, ensuring policies are being formulated in the right direction relies on strong communication channels between all of the layers of decision-making.

Thus far, both the problem identification and policy formulation stages have reflected on aspects of senior management buy-in and willingness to improve the decision-making process, which was also highlighted by the review. Ten of the 19 studies reflected on the role of senior leaders and highlighted their importance as a driver of allocating resources to wellbeing activities in both the public (Quirk et al., 2018) and private sectors (Nobrega et al., 2021). Strong senior leadership is considered a key enabler, with the role including giving HR managers resources, entrusting them to make decisions, and following up to assess effectiveness (Mellor and Webster, 2013). There needs to be "top-down", visible commitment from organisational leaders, employees must be aware of their support for wellbeing policies (Nobrega et al., 2021). If employees feel that managers are only superficially interested in wellbeing programs then they are "unlikely to become involved in, or support, organisational-health related initiatives" (Noblet and LaMontagne, 2009). Allowing staff from outside of leadership or supervisory groups to be involved in the decision-making process can accelerate this change and help tailor interventions to sections of the workforce (Badu et al., 2020). A lack of support and buy-in from leadership was described as the biggest barrier to resources being allocated to wellbeing schemes (Wieneke et al., 2019).

3.6. Stage 3a: Policy implementation (workforce)

Policy implementation is about turning the policy into practice and was a pronounced element in the case study and the systematic review. To turn the aspiration of mental health and wellbeing investment into a reality, a pre-requisite was workforce investment.

What seemed to be clear from our respondents was that the organisation knew they could improve their decision-making processes and invest in workplace wellbeing more effectively. Being unable to effectively evaluate wellbeing interventions represented an important blockage to investing in the 'right areas'. They overcame this by; firstly, hiring a senior individual to lead the wellbeing programme, and secondly by using them to collect better, more actionable, data. The key indicators were: feedback from the global survey and relatively low uptake of wellbeing interventions, with data showing that less than 50% of staff engaged with the EAP from July 2022 to January 2023 (from document 1).

"I don't think we had a clear direction. So my thing was we didn't have somebody running it. So we needed to invest in the wellbeing manager. That's one of the biggest investments that we've done, and that gives us a

clear, competent resource that can advise the business now which we've never had" [Head of EHS]

The wellbeing manager assumed the role of a critical friend to the business and took the next stage of the wellbeing strategy further by developing key performance indicators (KPIs) for the business. These were around supporting employee mental health; shifting the culture of workplace wellbeing, and more opportunities for engaging physical health.

"I joined the business this year and I have been developing these KPIs for the year based on what the Head of EHS created as part of the overall wellbeing strategy which came from the data that was collected between last July and January" [Health and Wellbeing Manager]

Another role of the Wellbeing Manager was to carry out health checks, which helped to solidify the understanding that more could be done to promote individual mental wellbeing. Moreover, these health checks presented another opportunity to collect routine data:

"Part of fulfilling some of the KPIs was by doing some on-site health checks which gives us another baseline of health metrics. So general metabolic risk factors, cholesterol etc" [Health and Wellbeing Manager]

As well as the Wellbeing Manager, 'bolt-on roles' with a specific focus such as wellbeing champions and mental health first aiders were developed. This expanded organisational capacity to implement and manage wellbeing initiatives effectively:

"Again, with our wellbeing champions, 2017–2019 that was a big buzzword, then COVID kicked in and mental health first aiders kicked in." [EHS Manager 4]

To enhance the effectiveness of these roles, there was a deliberate effort to move away from siloed approaches. Primarily this was through the formation of a new wellbeing steering group, where people making decisions across various departments came together to discuss what was happening in different parts of the business. Rather than working independently, individuals with wellbeing-related roles were now encouraged to collaborate with other professionals in the organisation and share their expertise:

"The steering group that we're now implementing, basically, it's made up of [...] ordinary people, wellbeing champions and first aiders, but also HR people and more senior managers. They [...] could be a voice for their department." [EHS Manager 3]

"They are already making decisions that have an impact on wellbeing ... So what we want to do is try and bring it all together so that we can support that." [Head of EHS]

3.7. Stage 3b: Policy implementation (delivery of initiatives)

In implementing wellbeing initiatives we identified three aspects as important to the stage: (i) culture shifts; (ii) staff engagement; and (iii) pilots. These are explained in more detail below.

3.7.1. Cultural shifts

Successful investment in mental health and wellbeing requires buy-in from all levels of the organisation. The systematic review identified senior leadership buy-in as a prerequisite for successfully embedding new wellbeing interventions into organisations. In the case study, this buy-in was nurtured through cultural changes that enable managers and line managers to actively support and champion mental health initiatives. The cultural shift comes through two key avenues: enabling managers through a change in leadership approach and embedding policies that hold managers accountable for employee wellbeing.

"It's part of your planning that you check you are doing what you planned on, so that you've got reviews for mental health and that's when you go

back to your key performance indicators and say we're doing this, is it doing what we wanted it to [...] and then that is where it becomes an embedded an embedded topic" [EHS Manager 3]

The incorporation of mental health considerations into performance indicators adds a layer of accountability for line managers. This involves evaluating whether the planned initiatives are achieving the intended outcomes and continuously refining strategies based on the results. By weaving wellbeing considerations into managerial processes, the organisation ensures that employee mental health is not a standalone initiative but an embedded aspect of day-to-day operations. This approach emphasises the ongoing commitment to employee wellbeing and reinforces the organisation's expectation that line managers actively contribute to this goal.

3.7.2. Engaging staff

A key step was to embed the wellbeing steering group in decision-making and start utilising their expertise to make better decisions. Although they were not involved in the final decision-making – which still rested with senior management – they would be used as a soundboard for ideas. Previously, decisions were made by senior management with some influence from survey data. There was a disconnect between decision-makers and those affected, but then, the process became more collaborative:

"It does tend to be very collaborative. But again, it's not a one-size-fits-all. It would be very topic-dependent, and we'll use the relevant forums and working groups to inform our decision-making." [EHS Director]

Having staff at all levels involved helped people know that investments are being made for them, or purposefully for people who are affected by health issues:

"So, if you want something to land and create a cultural shift is to create a story. So if you go and find out what the problem is, you then go and ask what the solution is, which we didn't use to do, we just used to give them a load of stuff (services). If you say 50% of you said you have trouble sleeping in a survey and then we did a working group and you said you would like this, then we launched an app, then people would know it was for them. Later we could tap back in and see how it benefits them – ask them what they think – then you close the loop and go again ... we need to show them "you said, so we did ..." [Head of EHS]

3.7.3. Pilots

Case study work provided valuable insights into how strategies were implemented in practice. These strategies for implementation ranged from extending small-scale pilots seen in the formulation stage involving a select group of individuals to full-scale rollouts affecting all employees or stakeholders:

"You begin to learn what works best for those types of audiences. So, it's a case of often piloting, trying, testing, getting feedback and then putting them into a toolbox that's available to the business" [EHS Director]

3.8. Stage 4: Policy evaluation

This stage refers to the monitoring and assessment of the success or failure of the policy and may lead to change or termination of the policy. It is a key stage in determining if the objectives are being met and how to invest further. In the review it is highlighted that in order to justify at a management level to invest beyond the statutory minimum, economic evidence can be needed (Jimmieson et al., 2021). In the case study, the senior management team (SMT) demonstrated a keen awareness that despite prior investments in wellbeing, improvements could be made by adopting a data-driven strategy and entrusting its delivery to an experienced individual. If managers are not being equipped with actionable data which gives them the knowledge and understanding of what needs

to be overcome then they will not invest effectively:

"For me, knowledge is good. If we are not giving the knowledge to the line managers then they won't invest, simple as that." [EHS Manager 4]

Evaluation can take various forms and often creates loops back to the other stages. In the case study, however, this did not happen effectively initially because of a lack of actionable data (see stage 3A), but once addressed, evaluation was manifested through assessment of uptake as well as through business cases, outcome monitoring and sometimes through intuitive arguments, rather than 'hard data'.

3.8.1. Uptake evaluation

In the case study work, this stage involved assessing the outcomes of implemented policies and strategies in addressing identified problems and achieving desired objectives. This evaluation step was comparatively easier for some types of interventions, one example of this the EHS manager provided was an app. With apps and employee assistance programmes, routine usage data can be collected and to enable decision-makers to decide whether based on the update the interventions offer value-for-money. For organisation-wide interventions, for example targeted at improving openness, it was harder to define effectiveness and uptake.

In cases where programmes are being reported as being 'effective' in improving employee wellbeing, the loop will be effectively considered closed. This could be in terms of cost-effectiveness; however, it is more likely to be based on the extent to which the intervention satisfies the goals of the organisation. Given an emphasis on continuous improvements, feedback was still incorporated to make the strategy better. Thus investment involved iterative cycles of evaluation, adjustment, and implementation to optimise organisational decision-making effectiveness over time. Conversely, when programmes and strategies were not acceptable or cost-effective decision-makers could return to the policy formulation stage. Here, they either reformulated the intervention or moved forward with a new strategy.

3.8.2. Feedback forums

As identified in the policy formulation stage, using steering (or feedback) groups was a key way to understand what staff need and want. Similarly, the evaluation stage will also utilise these key members of staff who are involved in wellbeing to understand how the interventions have been received by the workforce. In most cases, this represented a quicker, but still reliable, way of canvassing opinions on new initiatives, compared to surveys.

"As a business, you discuss some of the actions we take as a business and go and discuss with your consultative forums, take feedback, and have some very good interactive discussions. There's a hierarchy of control for how those consultative forums work as well. So, we've got that very good kind interactive way of consulting with the employee forums outside of surveys as well, where we can get some very good feedback." [EHS Director]

3.8.3. Intuition

The systematic review indicated that workplace investment decisions could often be based on intuitive arguments (Miller and Haslam, 2009; David et al., 2021). Similarly, in the case study, it was seen as a decision-making environment where senior leaders were trusted and this meant decisions could be made without formal assessments and relying on intuition. One example occurred during the COVID-19 pandemic where they acted on intuition and developed the idea for a well-being festival:

"The well-being festival was literally, I had this idea to do it, I rang my boss, who is the CEO. I said "I want to do this, I've got the budget. Is that right? Yes." That's kind of sometimes the sum total of how we do things. So, you know it's a very trust-based decision-making process. At a certain

level, I'm not expected to justify my decisions necessarily to my peer group. There's an understanding that if I'm saying it's a good thing to do. It's a good thing to do." [EHS Director]

4. Discussion

In this study we conducted a systematic review and case study to investigate why and how workplaces invest in mental health and wellbeing. The results suggest workplaces invest because they regard employee wellbeing as intrinsically important, but also to contain workplace costs and boost productivity, promote the workplace's image and due to legal responsibilities. Senior level buy-in is seen as an important prerequisite for wellbeing investments but commitments need to be visible to the workforce. The review and case study work highlighted how employees at all levels were involved in the decision-making process. By doing this, organisations helped to foster an environment where employees recognise the rationale behind investments and create a story to generate employee participation with interventions. The findings from the review and case study were combined and developed into a causal process theory. This study shows that policy stages in mental health and wellbeing investment revolve to a large extent around organisational buy-in, workforce investment and continual evaluation.

Generally, the systematic review and case study findings complemented one another with overlapping findings, but there were some areas where this empirical work offered significant detail not present in the prevailing literature in the field. In particular, although the establishment (or identification) of co-ordinating groups was identified as an important step (Noblet and LaMontagne, 2009), our case study work has suggested that the priorities and goals for employee wellbeing are set by the senior management, rather than involving the coordinating groups. We then saw a more collaborative process adopted in the case study, through staff surveys, conversations and health checks. This aspect of crafting a narrative through the adoption of a "you said, we did ..." approach remains underexplored in the existing literature.

The focus on addressing the lack of actionable data reflects the organisation's broader goal to become more data-driven and evidence-based in all decision-making processes. This aligns with early literature highlighting the importance of evidence-based decision-making in organisational management (Cyert and March, 1963). More importantly, it is arguable that intuitive decision-making can only take workplaces so far and typically produce satisfactory levels of wellbeing. However, to improve employee wellbeing more optimally, data-driven decision-making is needed.

The refined policy stages heuristic adapted for workplace decision-making operates differently from the standard model used in policy studies, for two main reasons. Firstly, unlike the linear process often depicted by policy models, workplace decision-making processes are not linear (Grandori, 2001). Decisions often loop back and forth between stages, particularly as new information is gathered, and stakeholder input is used to consider alternatives. This is evident at the evaluation stage, where decision-makers look at the data and decide if uptake is sufficient or could be improved. The iterative nature of our model allows decision-makers to be more flexible and responsive to changing organisational needs and external factors. Secondly, the heuristic places more emphasis on organisational culture and inherent workplace dynamics. By considering the unique culture, dynamics, and structures of organisations, decision-makers can be more aware of how decisions are made within the context of power dynamics, communication channels, decision-making hierarchies, and organisational norms. It is important to understand how these dynamics may influence the implementation and outcomes of policies and strategies. The deviations of our workplace MHWB investment model from rational choice is due to several key factors inherent to organisational contexts, especially employee engagement and participation. Unlike some health investments that a

national government may make for its citizens such as funding a new drug, workplace MHWB investment emphasises active involvement from employees. Similarly, there is a focus on meeting organisational goals, as organisations prioritise performance metrics. In summary, the adapted Policy Stages Heuristic aims to reflect the complexity and challenges inherent to organisational contexts.

There are several areas of potential insight and challenge from this work for conducting health economics research to support workplaces to make equitable and efficient health and wellbeing investments. First, the key role of leadership within the organisation and whether they buy into the idea that an important goal is mental health improvement and that it is legitimate to redirect internal resources to activity to support this. Second, to build capacity to invest and set priorities for mental health, this required a number of stages including better data collection and staff investment. Third, the relatively limited role of evidence (and certainly economic research evidence) in decision-making about committing resources was highlighted. There was little discussion of specific interventions and although 'business cases' were mentioned when committing resources for additional provision the implication was that internal data and intuitive arguments often swayed decisions more than external research evidence. Fourth, decision-making was definitely not simple or health-maximising but reflected the influence of a complex range of goals and constraints around legal duties, business reputation, and worker retention and productivity. Reflecting such goals in research evidence may enhance the transferability of the evidence produced but inevitably brings challenges too.

The combination of literature and case study was essential in achieving both a broad understanding of workplace mental health investment motivations and an in-depth examination of specific decision-making processes. The systematic review provided a theoretical framework, identifying initial mechanisms and widely relevant factors influencing investments in mental health and wellbeing, such as legal compliance, productivity, and corporate image. These mechanisms were then tested, refined, and contextualised through the case study, which highlighted the unique influence of organisational culture and decision hierarchies. Notably, where literature suggested abstract motivations like compliance and image, the case study elaborated on how these motives were operationalised in real-world settings, sometimes revealing contradictions between theoretical expectations and practical applications. For instance, while theory often stresses data-driven decisions, the case study revealed reliance on intuition and internal buy-in as equally critical factors. Thus, the systematic review established an initial set of mechanisms, while the case study allowed for the refinement and contextual validation of these mechanisms, illustrating how both methods converge to provide a comprehensive, layered analysis of the decision-making processes involved in mental health investments.

This study aimed to combine the strengths of systematic review (for breadth) and case study (for depth). Nevertheless, there are some limitations. In the case study, we intended to conduct field visits for some in-person interviews and observe meetings to understand how the issues of decision-making and resource allocation were discussed internally. However, post-Covid, people were rarely on-site at the same time and online meetings were seen as more pragmatic for evidence gathering. Similarly, we intended to collect more documents from the case study to triangulate our findings but the documents available from the organisation mostly focused on data collection for uptake rather than evidencing the decision-making process. Our conversations primarily involved individuals directly engaged in the decision-making process, including senior leaders and wellbeing representatives. It is worth noting that including more perspectives from individuals less directly involved in wellbeing initiatives, such as those outside the wellbeing sphere, might offer different insights or understandings of the process.

It is also worth reflecting on how the methodology of process tracing can best-fit questions related to understanding and improving resource allocation practice. Typically process tracing will trace a specific policy outcome. However, resource allocation practice has a somewhat fuzzy

endpoint (representing a collection of individual investments of money and time) and complex micro-processes associated with this. Further research to (i) conceptualise different resource allocation practices and outcomes, as well as (ii) apply theory-testing process tracing (Beach and Pedersen, 2013) to understand the factors that might lead to the causal mechanism proposed here to break down would be valuable.

Finally, as the review was formative for the case study work, some time has passed since we ran the initial search for the systematic review. This means the review element does not build on some potentially relevant studies focusing on aspects of planning (Haraldsson et al., 2024) and implementing (Benning et al., 2022) interventions that appear relevant. In the review, we included a systematic scoping review protocol (Paterson et al., 2021). The updated search identified the full study (Paterson et al., 2024) which has some overlapping objectives and findings with our study but primarily focuses on small and medium-sized organisations. Firstly, they found that continuous leadership buy-in at all levels of the organisation is important for the successful implementation of interventions. Moreover, organisations that lacked ownership of intervention content (due to a reliance on external consultants) typically struggled with long-term access to sustainable interventions. Conversely, champions within organisations facilitated smoother, more sustainable implementation.

In conclusion, this study has shown the organisational stages a workplace appears to go through when investing in mental health and wellbeing. Further process tracing work could involve ‘theory-testing’ process tracing in different contexts such as other industries, sectors (public, third sector) or countries. Nevertheless, the study’s emphasis on staff buy-in, workforce investment, optimising take-up and continual (and looping) evaluation presents challenges and opportunities for those conducting evaluative research to support resource allocation in these

settings.

CRediT authorship contribution statement

Luke Henstock: Writing – original draft, Methodology, Investigation, Formal analysis. **Rebecca Johnson:** Writing – review & editing, Validation, Supervision. **Philip Kinghorn:** Writing – review & editing, Validation, Supervision. **Derek Beach:** Writing – review & editing, Validation, Methodology. **Hareth Al-Janabi:** Writing – review & editing, Validation, Supervision, Methodology, Formal analysis, Conceptualization.

Ethics

Ethical Approval was granted in March 2023 by University of Birmingham’s STEM ethics committee. (ERN_22-1459).

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Appendix 1. Search strategy

Search terms reflecting four categories: (i) workplace setting, (ii) mental health and well-being focus, (iii) investment decision, and (iv) interventional focus, full search string:

- TITLE-ABS (((*employee*) OR (*employer*) OR (*workplace*))
AND ((*“mental health”*) OR (*wellbeing*) OR (*well-being*))
AND ((*investing*) OR (*invested*) OR (*investment*) OR (*resources*) OR (*budget**) OR (*allocat**) OR (*implementation*))
AND ((*intervention*) OR (*strategy*) OR (*strategies*) OR (*program**) OR (*initiative**) OR (*scheme**) OR (*promotion*)))
AND (LIMIT-TO (LANGUAGE, “English”))

Appendix 2. Key findings from studies included in the systematic review

Author(s)	Study Focus	Motives	Processes Identified (how workplaces invest in wellbeing)
McDaid (2007)	The economics of mental health in the workplace in the US and UK.	High economic costs for absenteeism and presenteeism. Employers need to maintain good working environments.	Not explicitly investigated.
Miller and Haslam (2009)	Interview study of information used in business cases for employee health activities in UK and US workplaces.	Ethical arguments ‘right thing to do’. Legal compliance and duty of care	Business cases for investment are not empirical (usually benchmarking or return on investment assessment). Scope for improvements in business cases through better measurements.
Noblet and LaMontagne (2009)	Discussion article on the challenges of developing, implementing and evaluating workplace wellbeing interventions (no specific country).	Not explicitly investigated.	Gain management support (engagement must be clear to staff), then establish/identify co-ordinating groups, conduct needs assessment, design interventions and action plan, and implement.
Attridge (2010)	Evaluating and selecting effective Employee Assistance Programs (EAP) for workplace behavioural health support in the US.	Not explicitly investigated.	Identifies and reviews employer guides for EAP. Reflects on the processes employers should go through when selecting EAP services, e.g. employing external staff.
Mellor and Webster (2013)	Identify enablers and challenges in implementing comprehensive employee well-being management in a large UK public sector organisation.	Managing employee occupational and lifestyle risks.	Mental health promotion requires strong senior leadership support, dedicated resources, involvement of stakeholders and intensive communication. Need for integrating health management systems with work systems (management of work demands, work pace, leadership style etc.)

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(continued)

Author(s)	Study Focus	Motives	Processes Identified (how workplaces invest in wellbeing)
Karanika-Murray and Biron (2015)	Deconstructing the elements of organisational interventions for stress and well-being, understanding why some interventions fail. Non-specific setting.	Not explicitly investigated.	Present the four pillars of organisational interventions for stress and well-being (context, content, process, and outcome). Most intervention studies focus on changes in target outcomes and remain silent on intervention conception, delivery process, or evaluation methods.
Fridrich et al. (2015)	Developing a context, process, and outcome (CPO) evaluation model for organisational health interventions (OHIs) in complex settings – general workplaces.	Not explicitly investigated.	Developed the CPO evaluation model for organisation wellbeing interventions. Unlike other models, it distinguishes the implementation process from the change process.
Chappell et al. (2016)	Investigate the implementation of workplace health initiatives using Kotter's change model in Australian organisations.	Background on the general rationale for promoting mental health but did not investigate the specific motivations for workplaces.	Formal change management models were not used – primarily testing if Kotter's Leading Change Model is relevant.
Chetty (2017)	Assessing employee health and wellbeing needs, resources, and perceptions in a British organisational context.	Identified and documented factors that were affecting the health and wellbeing of employees.	Normative - future initiatives should be developed considering the factors and resources that employees felt would be most beneficial.
Nogues and Finucan (2018)	Reviews economic evaluations of workplace mental health interventions, emphasising the impact of participation rates (USA, Germany, Japan).	Not explicitly investigated.	Barriers to increasing employee uptake and engagement – include reducing stigma and non-disclosure before investing in products targeted at individuals.
Quirk et al. (2018)	Qualitative study to understand barriers and facilitators to implementing health and wellbeing services in the NHS (England).	Not explicitly investigated.	Wellbeing investment requires a supportive culture; time; and financial incentives to invest. Barriers to implementation of services include a busy and pressurised environment, financial constraints and reluctance to invest in staff health and wellbeing.
McDaid et al. (2019)	General overview of the importance of mental health investment in the workplace.	Clear benefits to employers, positive return on investment and productivity gains.	More collaboration is needed between policymakers and the private sector to facilitate rigorous economic evaluations for interventions to support evidence base.
Walker et al. (2019)	Outlines Public Health England's approach to national public mental health initiatives (covers several key areas including workplace strategies).	Promote wellbeing, prevent issues, support recovery, ensure equity, reduce inequalities.	Integrate policies, engage stakeholders, use evidence, monitor outcomes, map pathways.
Wieneke et al. (2019)	Qualitative study designed to understand the supervisor's role in employee wellbeing in the US.	Cost containment but there was also the belief that mental health was an individual responsibility.	Recommend training, facilitate discussions, flexible schedules, developing toolkits. Identified barriers and facilitators for supervisors developing programmes.
Badu et al. (2020)	Review synthesises stress and resilience factors among Australian nurses, emphasising individual and organisational strategies.	Not explicitly investigated.	Staff from outside of leadership or supervisory groups need to be involved in the decision-making process – this can facilitate more acceptable decisions.
Jimmieson et al. (2021)	Supervisor strategies, challenges, competencies, and resources in managing employee stress in Australian organisations.	Legal duty of care and compliance.	Legal and compliance justified a minimum standard of intervention, beyond this economic evidence is needed.
Daniels et al. (2021)	Systematic review identified critical success factors for workplace health and psychological wellbeing implementation (in Europe, Canada, South Korea and China).	Not explicitly investigated.	Three key factors are associated with successful implementation: continuation, learning, and effective governance. Gaps in the evidence - suggested more research around organisational structures and contexts.
David et al., (2021)	Narrative review summarises initiatives developed during the COVID-19 pandemic to support healthcare workers' emotional well-being (China, UK, US, Spain, Hong Kong, France, Canada, Malaysia, Iran, Australia).	Not explicitly investigated.	Anecdotal evidence was used to inform the decision (possibly due to the COVID-19 situation). How the pandemic hindered the decision-making environment.
Schall and Chen (2021)	Review of practical evidence-based strategies that could be implemented for teleworkers wellbeing in US.	Not explicitly investigated.	Developed evidence-based strategy and detail how leaders should act as role models in the way they practice health and safety (including MH).
Nobrega et al. (2021)	Mixed methods study to understand the implementation of Total Worker Health (TWH) programmes (encompasses safety, physical and mental health).	Not explicitly investigated.	Lack of staffing is highlighted as a barrier to investment. Interventions must match the organisational context (due to complex social systems in workplaces).
Wu et al. (2021)	Narrative review of best and promising practices for achieving exemplary mental health in the workplace.	Wellbeing awards can incentivise organisations to improve employee wellbeing and encourage a positive workplace mental health culture.	These awards can also help workplaces identify evidence-based programs for staff wellbeing.

Appendix 3. Interview topic guide: well-being investments in workplaces

This topic guide provides an outline of the key issues and subtopics to be explored with participants in this study. We want to understand how and why mental well-being investment decisions, by asking questions about the topics below:

Topic 1: Establishing context (to cover your role(s), and experiences, including asking you about your role and the role of others in decision-making)

Topic 2: Mental health and wellbeing investments (types of programmes, staff roles (champions), infrastructure, policies, interventions in your organisation and how this varies across sub-organisations).

Topic 3: Decision-making steps (how are decisions about mental health and well-being investment made in your organisation?) (Discussion centred on issues related to meetings, budgets, strategy, consideration of options, consultation, implementation, staffing implications, the role of senior leaders, and evaluation of mental well-being investment).

Topic 4: Influences on well-being investment (e.g., understanding the roles of employees, managers, and stakeholders, as well as wider factors such as regulations, business cycles, corporate social responsibility, Covid-19, and health services).

Topic 5: Use of evidence and information We will ask you about the types of information used in decision-making in relation to mental health and well-being resource allocation (e.g., experiences, research evidence, business cases, demands from staff, government policies and regulations).

Topic 6: Further issues and conclusion (to include discussion about future contact, other participants, and any documents that may be helpful in understanding the chain of events in your organisation).

Data availability

Data will be made available on request.

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